

Audio course on steroids during pregnancy. Episode ten. Integrated cases and bedside counseling.

Companion reading handout for simultaneous listening.
This final episode turns the course into a bedside algorithm.

The core question is no longer, are steroids safe in pregnancy?

The core question is this: who is the target, which steroid reaches that target, why now, what dose and duration, what disease risk are we preventing, and what monitoring protects the mother and newborn?

Let us walk through cases.

Case one. Threatened preterm birth at thirty weeks.

A patient has regular contractions, cervical change, intact membranes, no infection, and reassuring fetal status. Delivery in the next week is plausible.

The target is the fetus. The steroid should cross the placenta. Betamethasone twelve milligrams intramuscularly, repeated once after twenty four hours, is appropriate. Dexamethasone six milligrams intramuscularly every twelve hours for four doses is the alternative regimen.

The counseling is direct: this steroid is meant to reach the baby. It reduces complications of prematurity, especially breathing problems, and the benefit is strongest when birth happens in the next week. We will monitor glucose, and if the baby is born soon, the neonatal team will watch blood sugar after birth.

Resident reflex: fetal target, fetal-transfer steroid, seven-day logic.

Case two. Chronic prednisolone with new preterm birth risk.

A patient with lupus takes daily prednisolone and now develops PPROM at thirty one weeks.

The resident asks, has she already received steroids?

Not for fetal lung maturation. Her prednisolone is maternal disease therapy and much of it is inactivated by the placenta. If preterm delivery is likely and no contraindication exists, she may still need betamethasone for fetal lung maturation.

The counseling is: your daily prednisolone helps control your disease, but it does not reliably count as the fetal lung-maturation course because the placenta breaks down

much of it before it reaches the baby. If birth looks likely, we use betamethasone because it is designed to reach fetal tissues.

Resident reflex: maternal prednisolone is not fetal lung maturation.

Case three. Lupus flare at ten weeks.

The patient has active lupus symptoms early in pregnancy. The family is worried about steroids.

The target is maternal disease control. The goal is not fetal lung maturation. The preferred logic is a lower fetal-transfer steroid such as prednisolone or prednisone when systemic therapy is necessary, plus a plan for pregnancy-compatible maintenance treatment.

Counseling should be balanced: some studies have raised a possible small signal for oral clefts with first-trimester systemic corticosteroids, but most human data are reassuring. More importantly, uncontrolled lupus can be dangerous for both mother and pregnancy. The goal is the lowest effective dose and a disease-control plan that avoids repeated high-dose rescue.

Resident reflex: do not let fear of medication create uncontrolled maternal disease.

Case four. PPRM at thirty two weeks.

The patient has ruptured membranes, no fever, no uterine tenderness, reassuring fetal status, and no immediate need for delivery.

The target is the fetus, and steroids are appropriate from viable gestational age through at least thirty four weeks. Betamethasone is reasonable. A short time-buying strategy may be considered if needed to complete steroids and if no infection is present.

Now change one fact. She has fever, uterine tenderness, and fetal tachycardia.

Now the target changes. The clinical problem is infection. Delivery and antibiotics move forward. Steroid completion is no longer the organizing goal.

Resident reflex: in PPRM, steroids are appropriate until infection makes time dangerous.

Case five. Late preterm planned delivery with diabetes.

A patient is thirty four weeks and three days, singleton pregnancy, high likelihood of delivery within the week, no prior steroid course, and diabetes requiring insulin.

Late preterm betamethasone may reduce respiratory morbidity, but it increases neonatal hypoglycemia and worsens maternal glucose. The decision must include glucose planning. The local GDM protocol suggests total insulin may need to rise by about twenty to thirty percent for three to five days after betamethasone, with close monitoring and individualized adjustment.

Counseling should say: the steroid may reduce breathing problems after birth, but it can raise your glucose and increase the baby's risk of low blood sugar after delivery. If we give it, the glucose plan is part of the treatment, not an afterthought.

Resident reflex: in diabetes, the steroid order is incomplete without the glucose plan.

Case six. Severe preeclampsia at thirty four weeks.

A patient has severe-range BP, persistent neurologic symptoms, and worsening labs. Someone asks whether to wait for late preterm steroids.

The answer is no if waiting delays indicated delivery. Stabilize the mother. Treat severe BP. Give magnesium when indicated. Plan delivery. Steroids may be given if they do not delay necessary care, but the mother is declaring severe disease.

Counseling should be clear: at this point, the priority is your safety and the baby's safety. Steroids can sometimes help the preterm baby, but they do not make severe preeclampsia safe to continue.

Resident reflex: severe maternal disease is not a steroid-completion project.

Case seven. ITP requiring platelet optimization.

A patient has platelets of fifteen thousand and mucosal bleeding.

The target is maternal bleeding risk and delivery safety, not fetal lungs. Steroids can raise the platelet count. Intravenous methylprednisolone one point zero to one point five milligrams per kilogram per day in divided doses may be used when faster response is needed. Oral prednisone one milligram per kilogram per day, or sixty to one hundred milligrams per day initially, is another anchor, then taper to a safe platelet target.

Counseling should say: the goal is not to make the platelet count normal. The goal is to make it safe for you, for delivery, and for anesthesia planning if possible.

Resident reflex: in ITP, treat bleeding risk, not a number for its own sake.

Case eight. Adrenal insufficiency in labor.

A patient with known adrenal insufficiency arrives in labor. She takes hydrocortisone at home.

The target is the adrenal axis. Labor is stress. The resident should not simply continue home meds and forget the issue. A stress-dose hydrocortisone plan is needed, followed by postpartum taper or return to baseline.

Counseling should say: your body may not be able to make enough cortisol for the stress of labor, so we give hydrocortisone to replace that response and prevent adrenal crisis.

Resident reflex: replacement is not optional immunosuppression.

Now final synthesis of the course.

Steroids in pregnancy are safe or unsafe only after you define the clinical target.

If the fetus is the target, choose a steroid that crosses the placenta, usually betamethasone or dexamethasone, and use it in a timing window where birth is likely soon enough for benefit.

If the mother is the target, control the maternal disease. Prefer lower fetal-transfer steroids when systemic therapy is needed, use the lowest effective dose, and do not abandon pregnancy-compatible maintenance therapy just to avoid medication.

If the adrenal axis is the target, replace physiology and prevent crisis. Labor, hemorrhage, infection, and surgery are stress events.

Then ask the monitoring questions.

What happens to maternal glucose?

What happens to neonatal glucose?

Could fetal testing look transiently different?

Is fetal growth at risk from drug, disease, placenta, or all three?

Does the neonate need to know about prolonged or high-dose maternal exposure?

Is breastfeeding compatible, and does timing after high doses matter?

The resident reflex for the whole course is this: ask target, transfer, timing, dose, duration, disease risk, and monitoring every time.

When those seven questions are answered, steroids in pregnancy stop feeling chaotic. They become a precise clinical tool.